



Cognitive Impairment Screening and Management Policy

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RDBG)	Hospital Wide	Medical, Nursing and Allied Health

Policy statement

This Policy establishes the minimum standards for the screening, assessment and management of patients with a cognitive impairment and is aligned with the Australian Commission on Safety and Quality in Health Care Clinical Care Standards.

Links within document

- [Appendix I: Cognitive screening and assessment instruments](#)

Definitions

Cognition	The set of higher cortical brain functions that includes memory, thinking, planning, problem solving, orientation, comprehension, calculation, learning capacity, language and judgement/intuition.
Cognitive Impairment (CI)	An overarching term referring to deficits in one or more areas of cognition. It is acknowledged that the term CI is inclusive of a broader range of genetic, medical and acquired conditions and that it can affect people from any age. For the purpose of this policy and to align with “a better way to care” initiative, CI will be used to refer to patients with dementia and delirium. ^{1,2}
Dementia	A syndrome due to disease of the brain. The decline in cognitive function that affects memory, judgement, attention, language, and problem solving. Consciousness is not clouded. It is usually gradual, progressive and irreversible – leading to impaired function.
Delirium	An acute change in mental status characterised by a disturbance in consciousness, attention, cognition and perception that develops over a short period of time, usually over hours or days. It is usually reversible. It is a serious condition and should be treated promptly and appropriately.
Four A’s Test (4AT)	A brief screening tool designed for the detection of possible delirium as well as cognitive impairment. This tool has 4 separate domains which measure alertness, AMT4, attention and acute change/fluctuating course. The tool has a maximum score of twelve with a higher score indicating increased impairment.

Cognitive Impairment Screening and Management Policy

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Screening and assessment procedure

Cognitive screening and assessment allow for the detection and appropriate actions to support people with CI. CI (dementia and delirium) is common in hospitalised older people, who are at high risk of sub-optimal outcomes and adverse events, especially if their CI is not detected or identified. All at risk patients as defined by the inclusion criteria in this policy are to receive an initial Cognitive Impairment screen using the 4AT Rapid Delirium Assessment Tool and Single Question in Delirium (SQiD):

- During presentation to the Emergency Department (ED) or as early as possible within the admission journey
- Following direct admission to the hospital/services.

Refer to [Appendix I](#) for cognitive screening and assessment instruments.

Inclusions

- All patients over 65 years of age and Aboriginal patients over 45 years
- Patients who meet any of the following criteria:
 - Cognitive concerns raised by others
 - Staff concerns
 - Pre-existing CI
 - A current hip fracture
 - SQiD +ve
 - Severe medical illness
 - History of delirium
 - Neurodegenerative disease
 - Acute neurological condition.

Exclusions

- Obstetric patients at Bentley Health Service (BHS)
- Patients with an acute brain injury requiring Glasgow Coma Scale (GCS) observations.

Emergency Department

- ED nursing and/or medical staff and/or Care Coordination Team will use the 4AT and SQiD to screen patients for CI and record this on the *Emergency Department Triage Form*
- Clinical staff who complete CI screening will document the results in the iSoBAR handover for the admitting ward/service.

Screening and assessment procedure cont'd

Admitted patients

- Following admission to the ward/service nursing staff or delegated clinician of the treating team will complete the 4AT:
 - on the *Multi-disciplinary Comprehensive Care Plan (MCCP)* as soon as possible
 - on the *Electroconvulsive Therapy (ECT) MCCP* for patients receiving ECT
- Screening should be completed within the first 10 hours following presentation to the hospital/service
 - If the score is 4 or more the nurse/delegated clinician will refer immediately to the Medical Officer for further delirium assessment (to be undertaken within 2 hours) and management
 - If the score is 1-3 the nurse/delegated clinician will refer to the Occupational Therapist (OT) for further review (to be undertaken within 24hrs)
- If screening within outpatient department or community services is required, appropriate referrals made and documented in the healthcare record and *Treatment, Support and Discharge Plan (TSDP)* in Mental Health
- Any decline in cognitive functioning or a change in behaviour during the admission will be documented on the *Adult Observation and Response Chart (AORC)* as a core observation under 'New Confusion or Altered Mental State' (refer to **RPBG Recognising and Responding to Acute Deterioration Policy** and **RRAD in Mental State SOP**)
- This will prompt a further 4AT screen and documentation in the multidisciplinary team (MDT) care plan as part of the general assessment / care planning
- A cognitive re-screen should also be undertaken on patients who meet the inclusion criteria following a:
 - Ward transfer or bed changes on the ward
 - Patient/family or support person request
 - New risk factor identified
 - Sudden decline in cognitive functioning or behaviour
 - Post incident (e.g. fall)
 - Medical condition change
 - Weekly risk re-assessment on new MCCP for long term patients
- CI screening will be documented in the iSoBAR handover for all inter-ward transfers
- Assess patient capacity as per **Assessing Patient Capacity SOP**.

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Screening and assessment procedure cont'd

Ongoing assessment

Formal cognitive assessment is to be conducted by the MO, OTs or Psychiatric Team to facilitate assessment of CI. Tools/screening tests include, but are not limited to:

- Mini-mental state exam (MMSE)
- Mental State Examination
- Montreal Cognitive Assessment (MoCA)
- Confusion Assessment Method (CAM).

Alternate assessment tools available for Aboriginal or culturally and linguistically diverse (CALD) patients are:

- Kimberley Indigenous Cognitive Assessment (KICA) - the preferred cognitive assessment tools for older Indigenous Australians (Aboriginal peoples), particularly for those living in remote and rural areas, aged 45 years and older
- The Rowland Universal Dementia Assessment Scale (RUDAS) - designed to enable the translation of the assessment into other languages and to be culturally fair. It is recommended for use with those from CALD backgrounds communicating in their first language with the use of an interpreter.

An assessment will seek input from the patient or their family/carer, GP and other primary care providers about behavioural changes such as:

- Confusion
- Worsened concentration
- Agitation
- Restlessness
- Hallucinations and paranoia
- Sleepiness (including altered levels of consciousness)
- Changed communicative status
- Cooperation with requests
- Alterations in mood
- The treating medical team must be informed of all patients identified as having CI (via formal assessment) and consideration given to further medical assessment to identify delirium and any causative (reversible) factors. A treatment plan should be initiated as indicated and documented in the patient's healthcare record. Patients identified with a CI may be recognised and /or flagged as follows
 - care boards (CI/forget me not flower)/ the journey/white board. Except ED
 - iCM (if a Patient of Concern) and in the patient's healthcare record.

Management and prevention strategies

Patients with CI or patients who may be at risk of developing delirium (i.e. score 4 or above on the 4AT screening tool) will be provided a set of individualised interventions to prevent deterioration. Medical review must be completed and consider interventions, including:

- Medical and/or mental health review to identify underlying causes
- Referral to the Geriatricians or Clinical Psychiatry service (via eReferral where possible), depending on the severity of underlying delirium or dementia
- Correction of medical issues (such as dehydration, malnutrition and constipation)
- Review of medications
- Early referral to Pharmacy, OT, Physiotherapy, Dietitian and Social Work (as appropriate)
- Regular mobilisation with assistance provided as required
- Regular fall, dysphagia and pressure injury risk reviews and management plan updates
- Pain assessment and management
- Regular reorientation and reassurance
- Ensure patient is wearing hearing aids, glasses and dentures and they are in full working order
- Meaningful activities for stimulating cognition (identified through using person centred tool e.g. Top 5 What Matters to me, Sunflower Tool); engaging with significant others to provide what matters most (WMM record)
- Natural sleep remedies and promoting healthy bedtime routine
- Environmental modification and support, including minimising bed moves where possible and decreased stimuli for hyperactive delirium
- Utilise the 'Forget Me Not Program' volunteers if available
- Increase observations using the *Behavioural Observation Form (BOF)* and *Visual Observation and Rounding Chart* (Mental health areas) as required
- Utilise Assistant in Nursing (AIN) and General Health Companion booklet.

Refer to **Delirium Preventative Considerations (Appendix III of Delirium Management CPS)** and the **Cognitive Impairment Management Pathway**.

- Clinicians should discuss these interventions and collaboratively develop a care plan with the patient and their family/carer, provide a copy to patient/carer, as well as general educational information (e.g. Delirium and Forget Me Not Hospital Volunteer Program pamphlets) and document this in the patient's healthcare record
- The MDT will continue to be alert to patients displaying signs of CI interfering with their activities of daily living and must refer to the treating team and document these changes in the patient's healthcare record. Patients with delirium are to be observed utilising a *Behavioural Observation Form (BOF)* (General Health) or *Intentional Rounding Form / Visual Observation and Rounding Chart* (Mental Health) for acute changes in behaviour, cognition and thinking
- Non-pharmaceutical approaches such as verbal de-escalation should always be attempted first and used in conjunction with consented pharmaceutical management, before moving to restrictive practices like restraint to administer parenteral medication.

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Management of agitation in older adults with dementia or delirium

Refer to:

- **Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP**
- Management of Agitation in older patients' flowchart (appendix V of the **Delirium Management CPS**)

Avoiding the use of Psychotropic medicines

Psychotropic medicines have several serious adverse effects for older people and can worsen CI.

Psychotropic medicines should only be prescribed for patients who are distressed despite non-pharmacological strategies, such as patients whose behavioural or emotional state poses a serious risk of harm to themselves or others.

If non-pharmacological strategies are ineffective and a Psychotropic medicine is being considered:

- Refer to pharmaceutical guidelines before prescribing and consult psychogeriatrics for MCCP /Treatment, Support and Discharge Plan (community MH services)
- Discuss the choice of Psychotropic medicine, the risks and benefits, dosage, and duration with the patient and carer
- A comprehensive medication review should be undertaken with input from the ward/service Pharmacist
- Commence with a low dose and closely monitor the effect for three to five days before considering any dose increases
- Limit use for as short a period as possible
- Ensure a de-prescribing regime is considered. Include in the discharge summary (Notifications and Clinical Summaries [NaCS])
- Refer to Australian Commission on Safety and Quality in Healthcare [Psychotropic Medicines in Cognitive Disability or Impairment Clinical Care Standard \(2024\)](#).

Discharge

Planning for discharge should be in collaboration with the patient, their family/carer, treating team and ideally commence at admission.

Prior to discharge, the patient and their family/carer should receive:

- Referral to community services including discussion and written contact details
- Individualised care plan with strategies to manage diagnosed CI or how to prevent recurrence of delirium
- General CI educational information to patient and family/support persons and offer relevant patient information brochures
- Detailed medication information (name, dose, reason and duration)
- Discharge summary (NaCS) to GP /other primary care providers listing all:
 - Presence of CI and episodes/ duration of delirium
 - Correct diagnosis of CI with relevant investigations and test results
 - Ongoing treatments
 - Follow-up needed
 - Medicines, including details on those that were ceased or changed and a recommended plan for ongoing review/withdrawal and/or consumables or equipment
- Possible memory clinic referral or note on GP/other primary care provider discharge letter.

Patients discharged from the Emergency Department

If the patient is to be discharged from the ED and has a 4AT score between 1-3, without a previous diagnosis of CI, note the 4AT score in the discharge letter generated from the ED Information System (EDIS) and recommend GP or other primary care providers follow-up and referral to a Memory Clinic.

If there is known CI prior to discharge, ED medical staff must ensure it is not due to new onset delirium. If delirium is suspected, the patient is to be admitted and cause investigated.

If the patient is transferred to another hospital, their cognitive status must be included in the verbal and written iSoBAR handover with recommendations to re-screen cognition on arrival.

Compliance monitoring

A combination of RPBG COBRA audits and rolling audits performed by Cognitive Impairment CNC provide data for performance monitoring. Audit data is reviewed by the relevant governance committee (Comprehensive Care) and escalated where appropriate to The Tier 1 Executive Committee.

Facilitator

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Fiona Stanley Fremantle Hospital Group Cognitive Screening of the older person policy
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East Metropolitan Health Service (EMHS) Aboriginal Health Strategy Team (2021)

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Cognitive Screening of the Older Person Policy through consultation with RPBG Aboriginal Health Liaison Officers to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 431).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

References

1. Australian Commission on Safety and Quality in Health Care. Delirium Clinical Care Standard. Sydney: ACSQHC, 2021.
2. A Better Way to Care: Safe and high-quality care for patients with Cognitive Impairment or at risk of delirium in acute health services. Second edition. ACSQHC. 2019.

Bibliography

1. Schofield, I., Stott DJ, Tolson D, McFadyen A, Monaghan J, Nelson D. Woodford HJ. (2010). Screening for cognitive impairment in older people attending accident and emergency using the 4-item Abbreviated Mental Test. European Journal of Emergency Medicine 17(6): pp 340-2.
2. Australian Commission on Safety and Quality in Health Care. (2021) Caring for Cognitive Impairment 4AT. Sydney

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Delirium Management CPS](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition and Behaviour Standard Operational Procedure \(SOP\)](#)
- [Vital Signs NPS](#)
- [Falls Prevention and Management for Inpatients CPS](#)
- [Behaviours of Concern Policy \(Preventing, Recognising and Responding\) CPS](#)
- [Cognitive Impairment Management Pathway](#)
- [Special and Supportive Observations Policy](#)
- [Assessing Patient Capacity SOP](#)

Other

- Refer to MyLearning and the Take 5 Library for a range of training related to cognitive impairment.
- [ACSQHC Cognitive impairment resources](#) – consumer and clinician resources
- [Psychotropic Medicines in Cognitive Disability or Impairment Clinical Care Standard \(2024\)](#)

Related forms

- *Emergency Department Triage Form*
- *Multi-disciplinary Comprehensive Care Plan (MCCP)*
- *Electroconvulsive Therapy (ECT) MCCP*
- *Treatment, Support and Discharge Plan (TSDP)*
- *Adult Observation and Response Chart (AORC)*
- *Behavioural Observation Form (BOF)*
- *Visual Observation and Rounding Chart*
- *Intentional Rounding Form*

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance;

Standard 2: Partnering with Consumers;

Standard 5: Comprehensive Care;

Standard 6: Communicating for Safety;

Standard 8: Recognising and Responding to Acute Deterioration

ACSQHC Delirium Clinical Care Standard 2021

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Appendix I: Cognitive screening and assessment instruments

Single Question in Delirium (SQiD)	A simple prompt question which asks a knowledgeable informant (usually a family member or support person), “do you think (patient’s name) is more confused, sleepy or drowsy lately?” If answer is Yes, patient is identified SQiD positive which may indicate a delirium and patient should then be assessed using the CAM Intensive Care Unit (ICU) (see below).
Confusion Assessment Method (CAM) ICU	A validated diagnostic tool that allows trained clinicians to diagnose delirium (adapted for RPBG).
Mini Mental State Examination (MMSE)	A screening test for evaluating CI in older adults. It tests personal information, orientation, short-term memory, attention, naming, visuospatial and language domains. A score of less than 24/30 suggests CI.
Rowland Universal Dementia Assessment Scale (RUDAS)	Designed to enable the translation of the assessment into other languages and to be culturally fair. It is recommended for use with those from CALD backgrounds communicating in their first language with the use of an interpreter.
Kimberley Indigenous Cognitive Assessment (KICA)	The preferred cognitive screen for older Indigenous Australians (Aboriginal peoples), particularly for those living in remote and rural areas, aged 45 and older.
Geriatric Depression Scale (GDS)	A depression assessment tool specifically designed for older people.
Montreal Cognitive Assessment (MoCA)	A validated screening assessment for detecting cognitive impairment. It is a 30-point one page test, administered in approximately 10 minutes
4AT – Rapid Clinical Test for Delirium	A screening tool for both delirium and cognitive impairment. The ‘months of the year backwards test’ and the ‘4-item Abbreviated Mental Test’ which are brief tests to cognitive impairment are also incorporated.